BASELINE → TREATMENT → FOLLOW-UP

Treatment checks across the course

Relative stages, not equally spaced dates. No patient dates are needed. Agree who arranges and reviews each check.

Check	Before starting	First induction dose	During induction	Maintenance review	After treatment
Bloods and hormones	FBC, kidney/liver, glucose, thyroid and hepatitis B/C screening	Review results and baseline symptoms	FBC, kidney/liver and glucose each cycle; thyroid every 3–6 weeks	Continue cycle bloods and thyroid checks; review symptoms	Follow unresolved abnormalities and the team's plan
Nerves, hearing and kidneys	Document baseline nerves/ hearing; renal assessment	Check cisplatin hydration	Nerve/hearing review before platinum; report new symptoms	Platinum stops; continue follow-up of any ongoing toxicity	Persistent symptoms need review
Support medicines	Folate/B12 lead-in and reconciled steroids	Use the induction support prescription	Check adherence, effectiveness and side effects	Continue folate/B12/rash prophylaxis; revise antiemetics	Folate 21 days after last pemetrexed
Immune effects and cancer review	Explain alert card, symptoms and treatment aims	Report infusion symptoms immediately	Report new cough, diarrhoea, rash or endocrine symptoms between visits	Continue pembrolizumab while appropriate; scans per consultant plan	Delayed immune effects remain possible

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00569 v5 · official regimen

THE WHOLE TREATMENT JOURNEY

Pembrolizumab + pemetrexed + cisplatin: treatment and checks

First-line metastatic non-squamous NSCLC without EGFR or ALK alterations

Induction: all three medicines on Day 1 every 21 days for up to four cycles. Maintenance: cisplatin stops; pembrolizumab and pemetrexed may continue every 21 days while appropriate. The team agrees duration and review points.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First induction dose	During induction	Maintenance review	After treatment
Treatment NCCP 00569	Confirm indication and planned phases	Three medicines on Day 1	Every 21 days; up to four induction cycles	Continue pembrolizumab + pemetrexed every 21 days while appropriate	Agree follow-up and next decisions
Bloods and hormones NCCP 00569	FBC, kidney/liver, glucose, thyroid and hepatitis B/C screening	Review results and baseline symptoms	FBC, kidney/liver and glucose each cycle; thyroid every 3–6 weeks	Continue cycle bloods and thyroid checks; review symptoms	Follow unresolved abnormalities and the team's plan
Nerves, hearing and kidneys NCCP 00569	Document baseline nerves/ hearing; renal assessment	Check cisplatin hydration	Nerve/hearing review before platinum; report new symptoms	Platinum stops; continue follow-up of any ongoing toxicity	Persistent symptoms need review
Support medicines NCCP 00569	Folate/B12 lead-in and reconciled steroids	Use the induction support prescription	Check adherence, effectiveness and side effects	Continue folate/B12/rash prophylaxis; revise antiemetics	Folate 21 days after last pemetrexed
Immune effects and cancer review NCCP 00569	Explain alert card, symptoms and treatment aims	Report infusion symptoms immediately	Report new cough, diarrhoea, rash or endocrine symptoms between visits	Continue pembrolizumab while appropriate; scans per consultant plan	Delayed immune effects remain possible

What each check is for

1 Before treatment

Before first dose

Establish suitability and baseline

Ask / act: Confirm results, support prescriptions and responsible teams.

Sources: NCCP 00569

2 Phase change

After up to four induction cycles

Active medicines and support change

Ask / act: Confirm maintenance medicines and revised support; continue immune monitoring.

Sources: NCCP 00569

3 After final treatment

Completion or treatment change

Delayed effects and continuing support

Ask / act: Give the follow-up and urgent-contact plan.

Sources: NCCP 00569

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-10-26 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.22